Ceritinib monotherapy

Advanced ALK-positive non-small-cell lung cancer (NSCLC) after crizotinib Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: NCCP 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Ceritinib daily oral treatment

Active medicines: ceritinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Individually selected daily oral antiemetic prophylaxis	Prescriber/pharmacy to select and prescribe under local oral-systemic anticancer therapy (SACT) policy	Daily while required during oral treatment; review prevention and rescue needs NCCP V6 Table 5 supports daily oral 5-HT3 antagonist prophylaxis for moderate-high risk. Ceritinib can prolong QT and cause bradycardia; check electrocardiogram (ECG)/ electrolytes and interactions before selecting an agent. Do not copy an infusion steroid/ NK1 bundle.

Additional phase-specific support

- Ceritinib has moderate-to-high oral sickness risk. Ask for daily prevention and rescue instructions. Your team must check heart-rhythm and medicine interactions when choosing anti-sickness treatment; do not add medicines or a steroid course yourself.
- If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.
- Avoid grapefruit or grapefruit juice and St John’s wort. Check prescription, pharmacy and herbal medicines before use, including heart medicines, antibiotics, antifungals, seizure treatments and acid-reducing medicines. Do not stop a prescribed medicine without advice.
- Discuss pregnancy, fertility and breastfeeding before treatment. Women who could become pregnant need highly effective contraception during treatment and for 3 months afterwards; hormonal contraception may be less effective, so agree a suitable method. Do not breastfeed during this treatment. Fertility effects are uncertain; ask about preservation if relevant.
- Use the agreed diarrhoea treatment promptly. Report persistent symptoms, poor intake or dehydration. Ask for help with appetite, fatigue and practical support; avoid driving or machinery if fatigue or visual symptoms affect safety.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00340 v4 · pNCCP 00340 v4 p5; classification Tables 5/8; current SmPC interactions · NCCP oral SACT antiemetic guidance V6

Sources checked 2026-10-29 · SACTCheck v0.84.25 · Regimen-level drafts; prescriber and oncology-pharmacy verification required